



**HKU
Med**

School of Clinical Medicine
Department of Family Medicine
& Primary Care
香港大學家庭醫學及基層醫療學系



COMMON MENTAL HEALTH PROBLEMS IN PRIMARY CARE



Prof. Amy Ng

Department of Family Medicine
and Primary Care

University of Hong Kong

amypng@hku.hk

(ONLINE)

Session Objectives

- To introduce students to the types of mental health problems which present to primary care with a focus on symptom identification and diagnostic approach



By the end of this session along with the in-person discussion, students will be able to:

1. understand the primary care approach to diagnosing patients with depression, anxiety, and undifferentiated psychological distress
2. recognize the relationship between psychological distress and unexplained physical symptoms
3. know some patient-centered strategies for assessing a depressed patient



The size of the problem

- Psychological symptoms are common in the general population affecting more than 50% of adults at some time¹
 - Worry
 - Tiredness
 - Sleep disturbance
- Approx. 1 in 7 may have a diagnosable mental disorder, the two most common being ^{1,2}
 - Depression
 - Anxiety Disorders (incl. generalized anxiety disorder, panic disorder, obsessive-compulsive disorder, PTSD, phobias)
- There is widespread under detection of depression and anxiety disorders by doctors (> 50% are missed) ³
 - Not all patients present with psychological symptoms, many present with somatic symptoms



1. Craig TKJ, Boardman AP. ABC of mental health: Common mental health problems in primary care. BMJ 1997; 314: 1609
2. Pilling S, Whittington C, Taylor C, Kendrick T. Identification and care pathways for common mental health disorders: summary of NICE guidance. BMJ 2011; 342: d2868
3. Chin, W.Y., Chan, K.T., Lam, C.L. et al. Detection and management of depression in adult primary care patients in Hong Kong: a cross-sectional survey conducted by a primary care practice-based research network. BMC Fam Pract 15, 30 (2014). <https://doi.org/10.1186/1471-2296-15-30>



Screening tools

- **PHQ2/PHQ9¹** and **GAD7²** are examples of **screening** instruments designed to help clinicians enhance the detection rates of common mental health disorders

These are NOT
diagnostic TOOLS.
It is used to
SCREEN ONLY



1. Spitzer R, Kroenke K, Williams J. Validation and utility of a self-report version of the PRIME –MD. JAMA 1999; 282: 1737-44
2. Spitzer R, Kroenke K, Williams J, Lowe B. A brief measure for assessing generalized anxiety disorder: the GAD7. Arch Intern Med 2006; 166:1092-7

PHQ-2 screen

Over the last 2 weeks, how often have you been bothered by any of the following problems?	0	1	2	3
Little interest or pleasure in doing things				
Feeling down, depressed, or hopeless				

Positive if cut off is ≥ 3

- sensitivity of 84%
- specificity of 90% for a major depression

0 = not at all
 1 = several days
 2 = more than half the days
 3 = nearly every day



PHQ-9 (Patient Health Questionnaire – 9)

- The nine items of the PHQ-9 are based directly on the nine diagnostic criteria for major depressive disorder in the DSM-V.
- Short and quick
- Can be administered in person by a clinician, by telephone, or self-administered
- Helps with the diagnosis of major depression
- Assesses of symptom severity and useful for monitoring treatment responses
- Validated
- Can be used in adolescents as young as 12 years of age



Over the last 2 weeks, how often have you been bothered by any of the following problems?	0	1	2	3
Little interest or pleasure in doing things				
Feeling down, depressed, or hopeless				
Trouble falling asleep, staying asleep, or sleeping too much				
Feeling tired or having little energy				
Poor appetite or overeating				
Feeling bad about yourself, feeling that you are a failure, or feeling that you have let yourself or your family down				
Trouble concentrating on things such as reading the newspaper or watching television				
Moving or speaking so slowly that other people could have noticed. Or the opposite - being so fidgety or restless that you have been moving around a lot more than usual				
Thinking that you would be better off dead or that you want to hurt yourself in some way				

0 = not at all
1= several days
2 = more than half the days
3 = nearly every day

PHQ-9 score interpretation

Total Score	Depression severity
0-4	No depression
5-9	Mild depression
10-14	Moderate depression
15-19	Moderately severe depression
20-27	Severe depression



PHQ-9 score interpretation

Total Score	Depression severity
0-4	No depression
5-9	Mild depression
10-14	Moderate depression
15-19	Moderately severe depression
20-27	Severe depression



PHQ-9 score ≥ 10 ¹ (for major depression)

- sensitivity of 88%
- specificity of 88%

1. Kroenke K, Spitzer RL, Williams JB. The PHQ-9: validity of a brief depression severity measure. J Gen Intern Med. 2001 Sep;16(9):606-13. doi: 10.1046/j.1525-1497.2001.016009606.x. PMID: 11556941; PMCID: PMC1495268.



Depression DSM V Criteria

- **Clinical depression** is diagnosed when there are at least 5 or more symptoms present for >2 weeks

1. **Low mood**
 2. **Loss of interest**
 3. Sleep disturbance
 4. Lost concentration
 5. Fatigue
 6. Change in appetite or weight
 7. Agitation/ retardation
 8. Feelings of worthlessness or guilt
 9. Suicidal thoughts
- Must have (1) and/or (2)



Depression DSM V Criteria

- Must have all 4
 - Symptoms cause significant impairment in social/occupation/other areas of function
 - Not attributable to physiological effects of a substance or other medical condition
 - Not explained by other schizophrenia/affective disorders
 - No history of manic/hypomanic episodes



Questions to assess the severity of depression

(adapted from Murtagh's General Practice 6th ed)

- What do you think is the matter with you?
- **Do you think that your feelings are possibly caused by nerves, anxiety or depression?**
- Can you think of any reason why you feel this way?
- Do you feel down in the dumps?
- Do you feel that you are not coping well? Or **How are you coping with this?**
- Do you have any good times?
- Has anything changed in your life?
- **How do you sleep?** Do you wake early?
- What time of the day do you feel at your worst?
- Where would you put yourself between 0% and 100%
- Have you felt hopeless?
- **What is your energy like?**
- **What is your appetite like?**
- Are you as interested in sex as before?
- Do you feel guilty about anything?
- Do you cry when no one is around?

Suicidal and Self Harm Assessment

- 45% of persons who complete suicide had consulted a GP within 1 month of their act¹
- Persons who survive lethal suicide attempts have similar clinical and psychosocial profiles as suicide completers²
- Over 50% of those who complete suicide initially presented with self harm²
- **Therefore, you should ASSESS for SUICIDE and SELF HARM in every patient you suspect depression**



1. Luoma JB, Martin CE, Pearson JL. Contact with mental health and primary care providers before suicide: a review of the evidence. *Am J Psychiatry* 2002;159:909–16.
10.1176/appi.ajp.159.6.909
2. Cullber J, Wesserman D, Lester D. Who commits suicide after a suicide attempt? An 8 to 10 year follow-up in a suburban catchment area. *Acta Psychiatr Scand* 1988;77:598–603.



GAD 7 (Generalized Anxiety Disorder – 7)

- 7-item scale, based on DSM-V criteria, for identifying likely cases of General Anxiety
- Validated



Over the past 2 weeks, how often have you been bothered by the following problems:	0	1	2	3
Feeling nervous, anxious or on edge				
Being unable to stop or control worrying				
Worrying too much about different things				
Trouble relaxing				
Being so restless that it is hard to sit still				
Becoming easily annoyed or irritable				
Feeling afraid as if something awful might happen				

0 = not at all
 1 = several days
 2 = more than half the days
 3 = nearly every day



GAD-7 score interpretation

Total Score	Level of Anxiety
0-4	No anxiety
5-9	Mild anxiety
10-14	Moderate anxiety
15-21	Severe anxiety



GAD-7 score interpretation

Total Score	Level of Anxiety
0-4	No anxiety
5-9	Mild anxiety
10-14	Moderate anxiety
15-21	Severe anxiety

GAD-7 ≥ 10 ¹ (for GAD)

- sensitivity of 89%
- specificity of 82%.

Can also be used as a quick tool to assess for other anxiety disorders



Generalized Anxiety Disorder DSM V

- Characterized by excessive anxiety and worry occurring more days than not for at least 6 months about a number of events or activities.
- The person finds it difficult to control the worry and it is associated with three (or more) of:
 - restlessness or feeling keyed up or on edge
 - being easily fatigued
 - difficulty concentrating or the mind going blank
 - irritability
 - muscle tension
 - sleep disturbance (difficulty falling or staying asleep, or restless unsatisfying sleep).



Generalized Anxiety Disorder DSM V

- Psychiatric symptoms must cause significant distress or significantly impair social or occupational functioning.
- Anxiety and worry cannot be accounted for by substance use or a general medical disorder
- **GAD 7 is commonly used screening tool for GAD**



How does mental health problems present in primary care?

Hey Doc, I'm feeling sad...

Hey Doc, I'm so stressed out!

I'm tired...

I have a headache...

I am short of breath

My body hurts like I have the flu...

I have dry mouth...

Bloating..



How often do patients have “medically unexplained physical symptoms”?

- In Primary Care – 18.5% of our consultations are MUPS
- “somatic”, “functional”, “unexplained”, “dissociative”
- Actively search for psychosocial stress
- Ask about sleep first
- If positive, ask about mood/anxiety



Cheung A et al. Psychosocial correlates of medically unexplained physical symptoms in primary care settings: a cross-sectional study in Hong Kong. HK Practitioner 2012



Insomnia

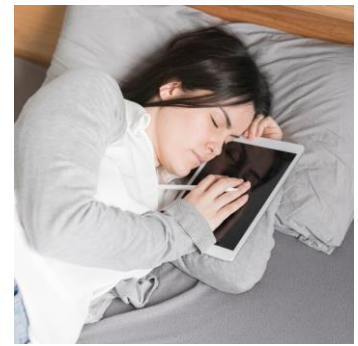
- 1/3 of patients experience occasional sleeping problems in primary care (AAFP)
- 10% adults experience persistent insomnia; Chronic insomnia refers to sleep difficulty occurring at least three nights per week for one month or more.
- Problems with sleep include
 - Difficulty falling asleep
 - Difficulty maintaining sleep
 - Waking up too early
 - Non refreshing sleep
- Consequences include:
 - Fatigue
 - Lack of energy
 - Difficulty concentration
 - irritability



<https://www.aafp.org/pubs/afp/issues/1999/0601/p3029.html>



Sleep Assessment



- How has the patient been sleeping recently?
- When did the problem begin? (To differentiate between acute and chronic insomnia)
- Does the patient have a psychiatric or medical condition that may cause insomnia? (May relate to an underlying condition that should be treated first)
- Is the sleep environment conducive to sleep? (Noise, interruptions, temperature, light)
- Does the patient report “creeping, crawling or uncomfortable feelings” in the legs that are relieved by moving the legs? (May relate to restless legs syndrome)
- Does the bed partner report that the patient's legs or arms jerk during sleep? (May relate to periodic limb movements in sleep)
- Does the patient snore loudly, gasp, choke or stop breathing during sleep? (May relate to obstructive sleep apnea)
- Is the patient a shift worker? What are the work hours? Is the patient an adolescent? (May relate to circadian sleep disorders/sleep deprivation)
- What are the bedtimes and rise times on weekdays and weekends? (May relate to poor sleep hygiene)
- Does the patient use caffeine, tobacco or alcohol? Does the patient take over-the-counter or prescription medications (such as stimulating antidepressants, steroids, decongestants, beta blockers)? (May relate to substance-induced insomnia)
- What daytime consequences does the patient report? (Daytime consequences may be significant)
- Does the patient report dozing off or having difficulty staying awake during routine tasks, especially while driving? (This is a serious problem that should be dealt with promptly)



How to assess a patient with depression

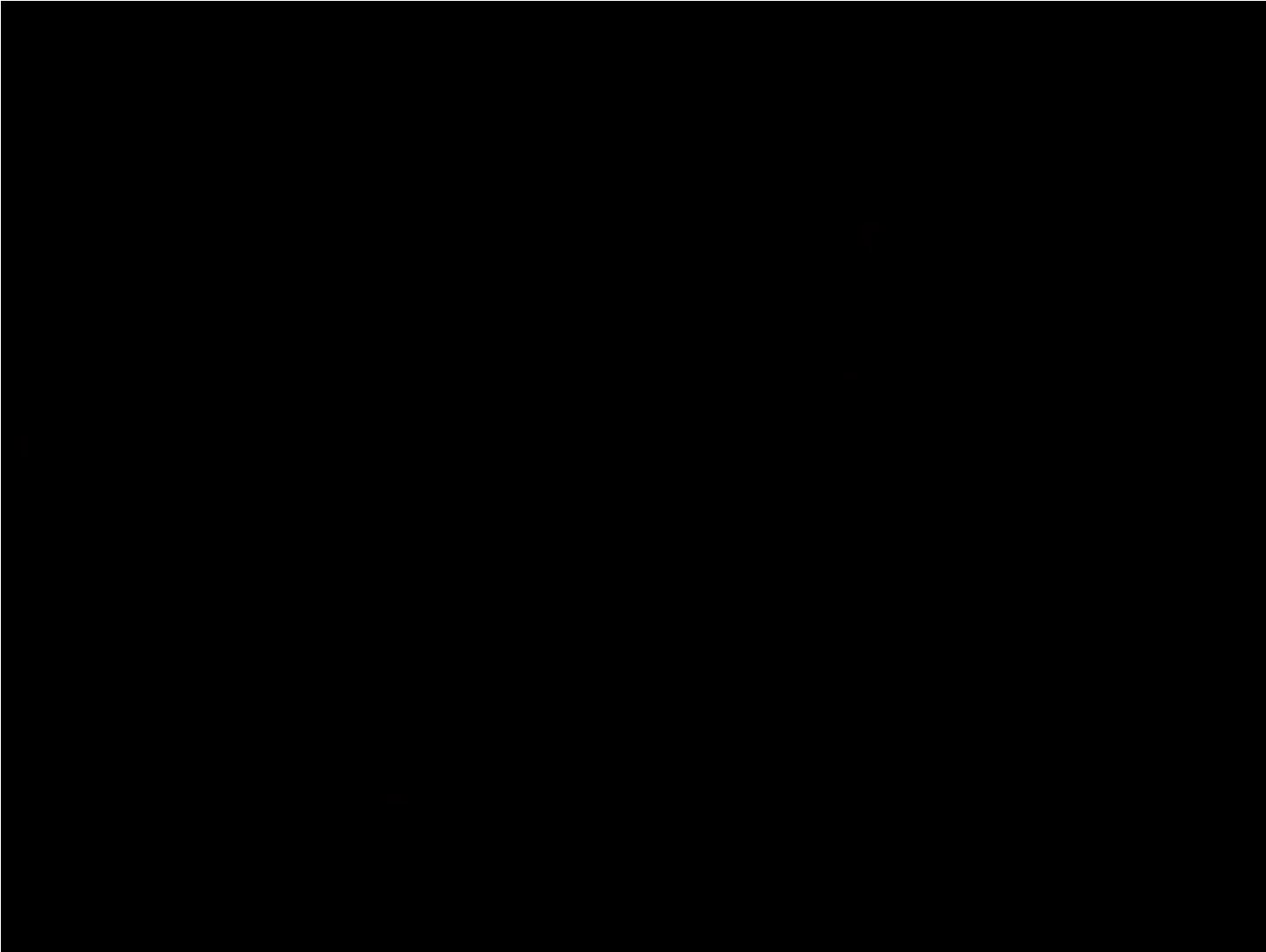


- Video excerpt from:

<https://www.youtube.com/watch?v=Gm3FLGxb2ZU>

- This video shows a mental health provider assessing a young male patient for depression and suicidality.
- Note the different ways to ask about depression to help the patient talk about his feelings.
- Males often will not express sadness, depression, crying, or hopelessness.
- Instead, they may experience frustration, anxiety, irritability, and anger, and may have somatic concerns.
- Asking about these symptoms often provides a pathway to talk more about their feelings, or to at least indicate their level of distress.





Clinician

“Why have you come in today?”

“What’s your mood like?”

“Are you feeling sad or depressed?”

“Have you been crying a lot?”

Patient’s response

Patients are often “sent in” by family

These are questions that men often do not relate to. Even if they are experiencing these feelings, they may not be able to identify them, or they may not want others to know they are experiencing them.

The patient is obviously confused about the mood question and denies feeling sad or depressed.

The patient is not interested in this line of questioning. He cannot discuss his sadness, even though he looks very sad.

Clinician

“Have you been feeling angry?”

“How are your energy levels?”

“How has school been?”

“Are you doing drugs/
alcohol?”

Patient's response

Males often experience sadness and depression as irritability and anger. The patient clearly becomes more engaged when he feels that the provider is asking him questions about the feelings he experiences.

The patient is much more comfortable talking about the physical aspects and somatic presentation of his distress.

The patient says his grades have dropped – but the Dr should have probed deeper to ask why his grades have dropped

The patient is obviously uncomfortable with this line of questioning and gives a very abrupt response. Sign posting may be helpful when discussing sensitive topics

Clinician

The Dr sums up what the patient has said, indicating that she has been listening closely and understands his concerns.

She looks calm when hearing about how much pain the patient is in, and this makes the conversation feel safe for the patient.

“How long have you felt this way?”

Patient's response

The patient feels connected enough to elaborate on what is bothering him, using his own words to describe his feelings.

The patient talks about anger, but looks very sad and discloses that he has had suicidal feelings.

Special Populations (Murtagh 5th edition)

Elderly

- Often misdiagnosed as dementia or psychosis
- May include odd behaviors such as histrionic behaviors, delusions, disordered thinking
- Assess sleep
- Helpful questions
 - Are you basically satisfied with your life?
 - Do you feel that your life is empty?
 - Are you afraid that something bad is going to happen to you?
 - Do you feel happy most of the time?

Perinatal

- Do not assume all women want to have children
- Even the ones who want to have kids can be sad
- Includes health professionals!
- Be non judgmental and remember to ask at every visit!
- Specific clues:
 - Poor weight gain in pregnancy
 - ++ anxiety /worries about parenting
 - Defaulting medical care
 - Low self esteem

Take home messages



1. Emotional symptoms are common but do not necessarily mean that the sufferer has a diagnosable mood disorder
2. Many mood disorders are short lived responses to stresses in people's lives such as work stress or bereavement
3. The cornerstone to the detection of a mood disorder is an understanding of the presenting symptoms and syndromes and exploring sleep, psychosocial history
4. Depression and anxiety are common in primary care but are missed by doctors in at least 50% of cases
5. Screening tools such as the PHQ-9 and GAD-7 can be useful to enhance detection rates for depression and anxiety
6. Patients with depression or anxiety often present with physical symptoms



THANK YOU!

For questions/ further information please contact me... and attend the WHOLE CLASS SESSION

Dr Amy Ng

Department of Family Medicine & Primary Care

E-mail: amyppng@hku.hk

